Clinic Prescribing and Medical Risk Guidelines

SAMPLE / FICTIONAL - for software testing only. Not medical advice.

1. Allergy check before prescribing

Always review the patient's medical alerts (allergies, conditions, current medications) before prescribing.
Penicillin allergy: do NOT prescribe amoxicillin, ampicillin or co-amoxiclav. Choose a non-penicillin alternative from the clinic formulary and record the reason in the notes.
NSAID cautions: avoid ibuprofen, diclofenac and aspirin in patients with gastric ulcer, kidney disease, asthma triggered by NSAIDs, or on anticoagulants. Paracetamol is the preferred first-line analgesic for these patients.
Metronidazole must not be combined with warfarin (it increases bleeding risk).

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2. Diabetes

Ask every diabetic patient for a recent HbA1c (within the last 3 months).

HbA1c below 7.5 percent: routine treatment may proceed.

HbA1c 7.5 to 8.5 percent: proceed with caution, prefer morning appointments, check glucose before treatment.

HbA1c above 8.5 percent: DEFER elective surgical procedures (extractions, implants, flap surgery) and refer the patient to a physician for glycaemic control. Emergency pain and infection management is still allowed.

3. Hypertension

If blood pressure is above 160/100 mmHg, defer elective treatment and refer to a physician. Limit adrenaline-containing local anaesthetic to the minimum effective dose.

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4. Pregnancy

The second trimester is the best time for necessary elective dental treatment. Avoid elective extensive surgery in the third trimester.

Avoid tetracycline-class antibiotics (tetracycline, doxycycline, minocycline) during pregnancy. Paracetamol is the preferred analgesic.

Obtain gynaecologist clearance before extensive procedures.

5. Radiographs in pregnancy

Take dental X-rays only when essential for diagnosis or emergency treatment. Always use a lead apron and thyroid collar, and record the justification in the notes.